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Answer

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Practice Exam - 1

Question 5 of 117



A 68-year-old man with a history of ACC/AHA Stage D nonischemic cardiomyopathy, heart failure with reduced ejection fraction (HFrEF), type II diabetes mellitus, hypertension, obesity (body mass index 34 kg/m²) presents to the emergency room with acute decompensated heart failure. He reports exertional dyspnea, orthopnea, and 9 lb. weight gain over the past 2 weeks. He has had two hospitalizations for acute decompensated heart failure for the past year. His medications include torsemide 100 mg twice daily, metoprolol succinate 12.5 mg daily, dapagliflozin 10 mg daily, sacubitril-valsartan 24-26 mg twice daily, spironolactone 12.5 mg daily, and metformin 1000 mg twice daily. Physical examination reveals blood pressure (BP) 90/65 mm Hg, heart rate (HR) 115 bpm, jugular venous pressure 12 cm H₂O, hepatjugular reflux, +S3 gallop, bilateral basal crackles, and 2+ lower extremity edema. Laboratories reveal sodium 132 mEq/L, blood urea nitrogen (BUN) 50, creatinine (Cr) 2.2 mg/dL, and potassium 5.2 mEq/L. Bedside echocardiography reveals left ventricular end-diastolic dimension 7.5 cm, ejection fraction 15%, moderate- severe centrally directed mitral regurgitation, and estimated right ventricular systolic pressure 40 mm Hg plus 15 mmHg right atrial pressure.

He is admitted to the cardiac intensive care unit and initiated on continuous infusions of furosemide titrated to 20 mg/hour and milrinone titrated to 0.375 mcg/kg/minute. Over the next 24 hours, his urine output is 2.5 liters but his BP decreases to 80/60. Furthermore, titration of milrinone results in frequent salvos of non- sustained ventricular tachycardia on continuous telemetry monitoring. Repeat labs demonstrate sodium 130 mEq/L, potassium 5.5 mEq/L, BUN 64, Cr 2.5 mg/dL. The patient has advanced directives, indicating he would want “reasonable measures to preserve a good quality of life.”

Which of the following is the most appropriate next step in his management?

- A. Refer for percutaneous mitral valve repair
-  B. Refer for right heart catheterization
- C. Refer for implantable pulmonary artery pressure sensor

D. Refer for initiation of continuous renal replacement therapy

E. Refer for palliative care consultation

Commentary

References

Testing Point: The Board-certified AHFTC specialist should recognize when to refer a patient for right heart catheterization.

Answer: B. Refer for right heart catheterization.

The patient has signs and symptoms of advanced heart failure with several high-risk features. These include but are not limited to recurrent HF admissions over the prior 12 months, inability to tolerate neurohormonal antagonists, hyponatremia, escalation of high-dose diuretics, and worsening renal function. Given that the patient has been refractory to initial challenge with high dose inotropes and intravenous diuretics, the next best step would be to pursue a right heart catheterization to characterize intravascular hemodynamics and facilitate pulmonary artery catheter guided therapy. This can also facilitate an evaluation for advanced therapies, including durable left ventricular assist devices and cardiac transplantation.

Answer A: Percutaneous mitral valve repair was shown to reduce morbidity and mortality in patients with disproportionately severe mitral regurgitation, including those with significantly less dilated ventricles on optimal medical therapy (as per the COAPT [Cardiovascular Outcomes Assessment of the MitraClip Percutaneous Therapy for Heart Failure Patients with Functional Mitral Regurgitation] Trial) and should not be used in patients with stage D HF requiring inotropic therapy in the acute setting.

Answer C: Pulmonary artery pressure monitoring has been demonstrated to reduce heart failure hospitalizations in patients with NYHA Class II or III regardless of ejection fraction (and this patient is NYHA Class IV).

Answer D: Initiation of continuous renal replacement therapy is not yet appropriate as the patient may benefit from oral or intravenous thiazide challenge, milrinone titration or transition to other inotropes/vasodilators pending the results of the right heart catheterization.

Answer E: while palliative care engagement and consultation may be reasonable, it may be premature to refer to hospice without consideration of further diagnostic testing given the patient's stated wishes and advanced directives.

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